

1. Administrative & Study Identification

Full Study Title: A Phase 1 Study of Sigvotatug Vedotin in Advanced Solid Tumors **Short Title/Acronym:** SGNB6A-001 **Protocol Number:** SGNB6A-001 **NCT Number:** NCT04389632 **Posting ID:** 199880777510017399 **Sponsor/Company Name:** Pfizer (Seagen, a wholly owned subsidiary of Pfizer) **Trial Status:** Recruiting (Currently recruiting participants) **Investigational Product:** Sigvotatug vedotin (SGN-B6A), pembrolizumab, carboplatin, cisplatin **Phase of Development:** Phase 1 **Medical Monitor:** Pfizer Clinical Trials Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and pharmacokinetics of sigvotatug vedotin, and to determine the maximum tolerated dose (MTD) and recommended phase 2 dose for participants with advanced solid tumors. **Secondary Objectives:** To assess the preliminary antitumor activity, including objective response rate (ORR), duration of response (DOR), progression-free survival (PFS), and overall survival (OS).

2.2 Background & Rationale Integrin beta-6 (IB6) is a tumor-associated membrane protein overexpressed in numerous solid tumors and serves as a negative prognostic indicator linked to tumor pathogenesis and invasiveness. Sigvotatug vedotin (SGN-B6A) is a novel antibody-drug conjugate (ADC) targeting IB6 to deliver a microtubule-disrupting agent, monomethyl auristatin E (MMAE), directly to cancer cells. This study aims to find a safe and active dose of sigvotatug vedotin—both as a monotherapy and in combination with immunotherapies like pembrolizumab—for patients facing relapsed, refractory, or advanced solid tumors.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose escalation and expansion cohorts) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 1,006 participants **Number of Sites:** Approximately 151 locations globally **Estimated Study Start:** 08-Jun-2020 **Estimated Primary Completion:** Jun-2027

4. Subject Selection (Eligibility Criteria)

- 4.1 Inclusion Criteria** 1. Age $\geq$ 18 years. 2. Histologically or cytologically confirmed metastatic or unresectable solid malignancy (including Non-small cell lung cancer [NSCLC], Head and neck squamous cell cancer [HNSCC], Advanced HER2-negative breast cancer, Esophageal squamous cell carcinoma [ESCC], Esophageal/Gastro-esophageal junction adenocarcinoma [EAC/GEJ], Cutaneous squamous cell cancer [cSCC], Exocrine pancreatic adenocarcinoma, Urinary Bladder Neoplasms, Uterine Cervical Neoplasms, Gastric/Stomach Neoplasms, or High grade serous ovarian cancer). 3. Disease that is relapsed/refractory to standard-of-care therapies (Parts A & B) or treatment-naïve for locally advanced/metastatic disease (Parts C & D).
- 4.2 Exclusion Criteria** 1. History of another malignancy within 3 years before the first dose of study drug, excepting malignancies with a negligible risk of metastasis/death. 2. Known active central nervous system (CNS) metastases. Patients with previously treated brain metastases may participate provided they are clinically stable for $\geq$ 4 weeks, have no enlarging metastases, and are off corticosteroids for $\geq$ 7 days prior to the first dose. 3. Grade $\geq$ 3 pulmonary disease unrelated to underlying malignancy, or a history of noninfectious interstitial lung disease (ILD)/pneumonitis that required steroids.
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5. Intervention & Treatment Plan

- 5.1 Investigational Regimen Dosage/Frequency:** Sigvotatug vedotin is evaluated at various weight-based regimens including 1.2/1.25 mg/kg total body weight on Days 1 and 8 of a 21-day cycle, or 1.8 mg/kg adjusted ideal body weight on Days 1 and 15 of a 28-day cycle. In Parts C and D, the drug is administered in combination with pembrolizumab (400 mg IV Q6W) $\pm$ carboplatin/cisplatin (ATC Code: L01XA01; Trade Name: Platinol). **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Until disease progression, unacceptable toxicity, or withdrawal of consent.
- 5.2 Concomitant & Prohibited Therapy Permitted:** Routine antimicrobial prophylaxis. **Prohibited:** Corticosteroids prescribed for symptoms associated with brain metastases within 7 days prior to the first dose.
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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Baseline tumor biopsy (fresh or archival), Imaging
Baseline	Day 1	First Dose of SGN-B6A, baseline safety assessments	Interim Cycles 1+ PK/PD sampling,

Safety Labs, Imaging for tumor response (RECIST v1.1), AE tracking | | **End of Study** | Variable | End of treatment assessment, Survival follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence of dose-limiting toxicities (DLTs), Treatment-Emergent Adverse Events (TEAEs), and laboratory abnormalities to establish the safety profile.

Measurement Method: Graded per NCI CTCAE v5.0.

7.2 Secondary & Exploratory Endpoints Confirmed Objective Response Rate (cORR) Duration of Response (DOR) Progression-Free Survival (PFS) evaluated per RECIST v1.1 Overall Survival (OS)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard TEAE monitoring, with specific attention to dyspnea, fatigue, neutropenia, and pneumonitis/ILD. **Serious Adverse Events (SAEs):** Protocol-defined evaluation and reporting timelines (e.g., within 24 hours for severe events). **Data Monitoring Committee (DMC):** Internal safety review committees evaluate safety data to authorize dose escalation and cohort expansions.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients who receive ≥ 1 dose of study drug); Efficacy Set (evaluable participants).

Sample Size Justification: A targeted enrollment of $N=1,006$ allows for adequate dose-escalation decision-making and robust expansion cohorts to detect preliminary efficacy signals.

Handling of Missing Data: Standard Phase 1 methodologies applied for missing tumor assessments and survival follow-up imputations.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular sponsor-directed onsite and centralized clinical monitoring. **Audit Trail:** Continuous review of validated electronic Case Report Forms (eCRFs) and data clarification processes.

11. Study Identifiers & Governance

Primary ID: SGNB6A-001 **Registry Number:** NCT04389632 **Posting ID:** 199880777510017399 **Sponsor/Lead Organization:** Pfizer **Study Title:** A Study of Sigvotatug Vedotin in Advanced Solid Tumors **Official Regulatory Title:** A Phase 1 Study of Sigvotatug Vedotin in Advanced Solid Tumors

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced Cancer / Advanced Solid Tumors (e.g., Adenocarcinoma, NSCLC, HNSCC, ESCC, EAC/GEJ, cSCC) **Disease Areas:** Carcinoma, Non-Small Cell Lung, Squamous Cell Carcinoma of Head and Neck, HER2 Negative Breast Neoplasms, Esophageal Squamous Cell Carcinoma, Esophageal Adenocarcinoma, Gastroesophageal Junction Adenocarcinoma, Ovarian Neoplasms, Cutaneous Squamous Cell Cancer, Exocrine Pancreatic Adenocarcinoma, Urinary Bladder Neoplasms, Uterine Cervical Neoplasms, Stomach Neoplasms. **Diagnosis Threshold:** Histologically or cytologically confirmed metastatic or unresectable solid malignancy **Medical Methodology:** Integrin Beta-6 (IB6) directed Antibody-Drug Conjugate (ADC) therapy **Optimization Target:** Finding the Maximum Tolerated Dose (MTD) / Tumor Response

13. Study Procedures & Methodology

Management Pathway: Dose Escalation (Part A) leading to Dose Expansion (Part B), followed by Combination Safety (Part C & D) **Education Protocol (HCP):** Investigator site training on ADC-specific toxicities and study protocols **Education Protocol (Patient):** Informed consent detailing risks, biopsy requirements, and treatment schedule **Quality Audit Mechanism:** Source data verification and continuous sponsor-level clinical site monitoring

14. Observation & Follow-Up Schedule

Total Duration: Follow-up until disease progression or completion of survival tracking **Onsite Visit Cadence:** Corresponds with 21-day or 28-day treatment cycles **Remote Monitoring Frequency:** Continuous safety oversight between dosing windows **Checklist Review Interval:** Routine symptom and concomitant medication checks per cycle

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				

Clinical Operations Lead | [Name] | ____ | [DD-MMM-YYYY] | | Lead
Statistician | [Name] | _____ | [DD-MMM-YYYY] |